HMI Client Contact Hours (CCH) Log

Student: Jithendran Sellamuthu
AHA #: 007913
Client Initials: T.M.
Session #: 3
Date: October 24, 2026
Duration: 90 minutes
Modality: Zoom (Virtual)
Fee: Pro Bono (CSP)

1. Presenting Issue

T.M. is a 22-year-old student at Appalachian State University in North Carolina, referred through the HMI Community Service Program (CSP). He presents with anxiety, hypervigilance, trust difficulties, and self-esteem challenges stemming from childhood trauma. This session significantly expanded the clinical picture: both parents were absent during early childhood (mother imprisoned for methamphetamine addiction, father incarcerated for heroin addiction). Client was passed between foster situations, some involving basic neglect. His grandfather provided relative stability (food, shelter) but worked extensively and was emotionally unavailable. Three uncles served as primary caregivers and were at times verbally and physically abusive. Client internalized the abuse as his own fault. The core wound was refined from the original formulation (abandonment/betrayal/shame) to **abandonment and alienation** — a pervasive felt sense of being an outsider, not belonging, and being fundamentally different from peers who had stable, emotionally available families. Suggestibility profile: 31% Physical / 69% Emotional.

2. What preparation did you do for this session?

- Reviewed Session 2a notes, SOAP notes, feedback form responses, and post-session observations
- Prepared a 17-phase Circle Therapy (Kappasinian fear extinction) verbatim readout with 4 confrontation cycles targeting: men yelling (low + medium intensity), vulnerability/being seen in group settings, and trusting someone close
- Prepared a self-esteem imagery script (Flip Theory: esteemable acts build self-esteem) with 5 workbenches, CRITIC-to-COACH dial installation, self-efficacy/self-worth gauges, and CBT 4-step rewrite embedded in trance imagery
- Prepared a client-facing Self-Esteem Toolkit (thought reframing template + somatic techniques: physiological sigh, 5-4-3-2-1 grounding, peripheral vision expansion)
- Prepared a merged session document combining Circle Therapy + Self-Esteem Script + Toolkit into one continuous flow
- Prepared SMART baseline collection protocol (6 scales, 1-10) deferred from Sessions 1 and 2a
- Reviewed HMI case conference guidance from John Melton (inner child), George Kappas (reprogramming vs story), Lisa Machenberg (wound identification), and Saman Nasir (neuroscience-informed rewiring)

3. What techniques did you use in this session?

No hypnotic induction or trance work was performed in this session. The session was entirely conversational: a deep check-in, psychoeducation, clinical history gathering, and collaborative treatment planning. This adaptation was clinically appropriate — Tristan arrived with significant new childhood disclosures that required careful processing before any trance work. Proceeding with the prepared Circle Therapy desensitization protocol without first understanding the full scope of the abandonment wound would have been premature and potentially destabilizing. The following conversational and psychoeducational approaches were employed:

- **Protector Energy Validation (check-in):** Client reported successfully applying the "protector energy" reframe from Session 2 in a real workplace situation involving angry park visitors. He maintained calm, offered help, and did not internalize their anger. This was affirmed as evidence of genuine internal character rather than a learned technique — a critical distinction for self-esteem work.
- **Root Wound Refinement (Socratic dialogue):** Through guided questioning, the core wound was refined from the Session 1/2 formulation (betrayal/shame) to **abandonment and alienation**. The client's discomfort watching peers connect emotionally in his Mankind Project group triggers a felt sense of being an outsider — a direct echo of childhood abandonment. The comparison to peers is driven by a need for belonging, not genuine aspiration toward specific traits.
- **Psychoeducation — Known vs. Unknown Principle:** Why the mind prefers familiar discomfort (emotional guarding) over unfamiliar safety (openness and trust). The nervous system defaults to what it knows. Applied to the client's pattern of shutting down emotionally even in objectively safe environments.
- **Psychoeducation — Cat-and-Boiling-Milk Analogy:** How a single painful experience of vulnerability in childhood can produce lifelong avoidance of openness, even when current conditions are safe. The client's guard activates at the first sign of intimacy regardless of whether the person is trustworthy.
- **Psychoeducation — Identity vs. Mimicry:** Risk of losing personal identity by copying peers' behavior purely for acceptance. "Pizza analogy": agreeing to eat something you dislike just to avoid exclusion. The goal is to become more fully himself without the protective walls that childhood required.
- **Psychoeducation — Dopamine, Goals, Comparison:** Dopamine release during pursuit (not achievement), comparison as a biologically normal motivational signal, and the critical filter: "Do I genuinely want this, or am I reacting to feeling excluded?" Fear of success as a real clinical pattern.

- **Psychoeducation — Boundary Maintenance:** "Homeostasis zone" — neither sacrificing self (people-pleasing) nor absorbing harm. Explaining limits calmly and walking away without escalation.
- **Phased Therapy Plan (collaborative agreement):** Phase 1 = conscious behavioral skills (emotional regulation, boundary-setting, social engagement via listener/helper role). Phase 2 = inner child work in trance once behavioral coping is demonstrated. Phasing explained: premature deep regression without external coping capacity risks destabilization.
- **Therapist Self-Disclosure (modeling):** Shared personal experience of being mislabeled as "shy" when actually experiencing social anxiety, and a same-day clinic encounter with an angry staff member, to normalize emotional reactivity and model non-reactive processing. Calibrated: brief, relevant, in service of the client's understanding.
- **Inner Child Work Preview (no trance):** Previewed the methodology for Session 4: adult self observes younger self from safe space, re-parenting, desensitization, dissolving the belief that vulnerability leads to punishment. Emphasized this was always part of the treatment design — the timing is intentional.

4. What were the highlights of this session?

- **Real-world application of Session 2 work:** The protector energy reframe was successfully deployed at the client's workplace with angry visitors. This is the strongest evidence of between-session integration across all three sessions. The therapist noted this level of emotional regulation "typically takes years" — reflecting the client's genuine inner character being unlocked rather than a technique being imposed.
- **Major childhood history disclosed:** Mother imprisoned (methamphetamine), father incarcerated (heroin), both absent. Foster situations with basic neglect. Grandfather provided stability but was emotionally unavailable. Three uncles were verbally and physically abusive. Client internalized abuse as his own fault — assumed his behavior caused the uncles' anger, though they never explained their reactions. This is the most complete picture of the client's childhood to date.
- **Root wound refined to abandonment + alienation:** The client's self-awareness breakthrough: recognizing that his discomfort in groups goes beyond surface-level social anxiety. It triggers the felt sense of being an outsider, a stranger, someone who does not belong. This is a direct echo of a childhood spent without consistent parental presence or secure attachment.
- **Self-reinforcing spiral identified:** Feeling alienated → withdrawing further → feeling more alienated. The client's guarded stance in the Mankind Project group, if unchecked, risks deepening the very isolation it was designed to protect against.
- **Listener/helper strategy co-created:** Rather than forcing vulnerability before the client is ready, the strategy is to build trust incrementally through service — focusing on others' problems, offering empathy and ideas, attempting small safe moments of openness only where comfortable. This aligns with both the esteemable acts framework and the Emotional lane (self-discovery over instruction).
- **Session adapted appropriately (third time):** As with Sessions 1 and 2a, the prepared script was set aside in favor of following the client's presenting material. The Circle Therapy and self-esteem imagery scripts remain ready for deployment when the client has sufficient behavioral foundation. This pattern of adaptation is consistent with the Emotional lane and with trauma-informed care principles.

5. Did you discuss this session with a supervisor and if so what guidance did you receive?

Prior to this session, I reviewed relevant case conference guidance from four HMI instructors:

- **Lisa Machenberg:** Three primary wounds model. The session confirmed abandonment as the primary wound (people who should have been present were not), with betrayal as secondary (uncles who should have protected him were abusive). The wound identification occurred organically through the client's narrative rather than the formal 3-wound presentation.
- **John Melton:** Inner child work must wait for appropriate readiness. Client must have sufficient external coping capacity before deep regression work. Room must be NEW (not from childhood). Applied: inner child work explicitly deferred to Session 4, pending client's demonstrated behavioral skills and journal input from Tuesday's group session.
- **George Kappas:** Address the PROGRAMMING, not the story. Therapy-experienced clients may intellectualize. Applied: moved from story-gathering to actionable behavioral strategy (listener/helper role, grounding intentions) rather than extended narrative processing. The childhood history was heard, validated, and contextualized, but the therapeutic focus shifted to present-tense behavioral change.
- **Saman Nasir:** Inner child = real neural survival maps in the limbic system. Insight alone cannot reach the limbic system — this is why the client's prior psychotherapy helped but did not fully resolve the patterns. Applied: the phased plan explicitly acknowledges that behavioral skills (Phase 1) create the container, and trance-based inner child work (Phase 2) accesses the limbic system to recode survival maps that talk therapy alone could not reach.

6. What do you plan to do for the next session?

- **Receive and analyze client's journal:** Tristan will attend his Tuesday Mankind Project group session, adopt the listener/helper role, and journal afterward: what felt blocked, what emotions arose, where the guard went up. This journal becomes the blueprint for the inner child script.
- **Collect SMART baselines:** Six self-report scales (1-10). Deferred from Sessions 1, 2a, and 3. Non-negotiable for Session 4.
- **Inner child work (Phase 2 begins):** Adult self observes younger self from safe space. Re-parenting: giving the child what was missing (safety, validation, "it was not your fault"). Wound-specific: abandonment ("you were always worth staying for") and uncle-related trauma (reframe their behavior as their dysfunction, not client's fault). Agency language throughout — client discovers what to say, therapist does not script the words.
- **Decision point:** Whether to use imaginary dialogue with uncles (receiving their acknowledgment/apology) or self-absolution approach (client absolves younger self of blame). The journal will inform which approach is more therapeutically appropriate.

- **Circle Therapy available as backup:** If inner child work feels premature after reviewing the journal, the full Circle Therapy protocol (4 confrontation cycles + reversal + compound anchor) remains prepared and ready for deployment.
- **Self-Esteem Toolkit:** Send to client as between-session homework (thought reframing template + somatic techniques).
- **Monitor:** Nightmare pattern, anchor usage, protector energy deployment, self-labeling frequency.
